

HOSPITAL DISCHARGE SUMMARY

Date (auto): 2026-03-18

Document ID: healthclarify-ips-en-01

PATIENT INFORMATION

Name John Smith
Age 64
Gender male

ALLERGIES

- Penicillin - diffuse maculopapular rash (documented 2018)

CLINICAL HISTORY

- Known hypertensive for 5 years on monotherapy; suboptimal blood pressure control in last 12 months.
- Type 2 diabetes mellitus diagnosed 8 years ago; on metformin 1000 mg BID.
- Active tobacco smoker (~30 pack-years); refused cessation previously.
- No prior myocardial infarction, stroke, or chronic kidney disease.
- Father died of sudden cardiac death at age 60.

LAB VALUES AT DISCHARGE

- Troponin I.cardiac [Mass/volume] in Serum or Plasma: 14200 ng/L (< 14)
- Cholesterol [Mass/volume] in Serum or Plasma: 245 mg/dL (< 200)
- Cholesterol in LDL [Mass/volume] in Serum or Plasma: 160 mg/dL (< 100)
- Cholesterol in HDL [Mass/volume] in Serum or Plasma: 32 mg/dL (> 40)
- Hemoglobin A1c/Hemoglobin.total in Blood: 7.8 % (< 6.5)
- Creatinine [Mass/volume] in Serum or Plasma: 1.1 mg/dL (0.6-1.2)
- Glomerular filtration rate/1.73 sq M.predicted: 68 mL/min/1.73m² (> 60)
- Left ventricular Ejection fraction: 45 % (> 55)

ADMISSION DIAGNOSIS

Acute transmural anterior ST-elevation myocardial infarction (STEMI)

SECONDARY DIAGNOSES

- Essential (primary) hypertension
- Type 2 diabetes mellitus without complications
- Mixed hyperlipidaemia
- Tobacco use disorder (current smoker, ~30 pack-years)
- Family history of premature coronary artery disease

PROCEDURES PERFORMED

- Primary percutaneous coronary intervention (PCI) via right radial approach
- Drug-eluting stent (DES) deployed in proximal left anterior descending (LAD) artery
- Post-PCI TIMI flow grade 3 restored
- Dual antiplatelet therapy initiated (DAPT: aspirin + ticagrelor)
- Cardiac rehabilitation Phase I begun before discharge

CLINICAL NARRATIVE

A 64-year-old male smoker with hypertension and type 2 diabetes presented to the Emergency

HOSPITAL DISCHARGE SUMMARY

Department with 90 minutes of severe substernal chest pressure radiating to the left upper extremity, diaphoresis and progressive dyspnoea on minimal exertion. ECG demonstrated ≥ 2 mm ST-segment elevation in leads V1-V4, consistent with an acute anterior ST-elevation myocardial infarction. The patient was taken emergently to the cardiac catheterisation laboratory. Coronary angiography revealed a 95% thrombotic occlusion of the proximal left anterior descending (LAD) artery. A drug-eluting stent was successfully deployed, restoring TIMI 3 flow. Peak high-sensitivity troponin I reached 14,200 ng/L. Post-procedural transthoracic echocardiography showed a mildly reduced left ventricular ejection fraction of 45% with hypokinesia of the anterior wall and the interventricular septum. Dual antiplatelet therapy was continued. The patient remained haemodynamically stable, without recurrence of chest pain or arrhythmias, and was deemed suitable for hospital discharge with structured cardiology follow-up and enrolment in the cardiac rehabilitation programme.

DISCHARGE MEDICATIONS

Medication	Dose	Purpose	Schedule
Acetylsalicylic acid	100 mg daily	Antiplatelet - prevents blood clots	Morning
Ticagrelor	90 mg twice daily	Antiplatelet - protects the new stent	Morning and evening
Atorvastatin	80 mg daily	Lowers cholesterol and stabilises plaques	Bedtime
Ramipril	2.5 mg daily	Protects the heart and lowers blood pressure	Morning
Metoprolol	25 mg daily	Slows heart rate and protects the heart	Morning
Metformin	1000 mg twice daily	Continuation of diabetes treatment	Morning and evening

FOLLOW-UP INSTRUCTIONS

Cardiology outpatient clinic in 14 days. Daily home blood-pressure log. Smoking-cessation counselling appointment within 7 days. Cardiac-rehabilitation Phase II starts in 3 weeks. Repeat lipid panel, HbA1c, and renal-function tests in 3 months.

WARNING SYMPTOMS — CALL 112 IF:

- ! Recurrent chest pain or pressure lasting more than 5 minutes
- ! Shortness of breath at rest or worsening at night
- ! Sudden dizziness, fainting or near-fainting
- ! Sudden swelling of the ankles or legs
- ! Bleeding that does not stop within 10 minutes (while on antiplatelet therapy)
- ! Any new neurological symptom (weakness, slurred speech, vision loss)

HOSPITAL DISCHARGE SUMMARY

This document is a synthetic, fully anonymised discharge summary created for HealthClarify demonstration purposes. Not a real medical record.